

Maternity Labs

CBC / Blood Labs

Lab	Basic Value	Nursing Cue
Hemoglobin	Pregnancy: usually ≥ 11 g/dL	Low can indicate anemia or blood loss. Watch fatigue, dizziness, pallor, tachycardia, and heavy bleeding.
Hematocrit	Pregnancy: usually $\geq 33\%$	Low can indicate anemia or hemorrhage.
WBC	Adult: 5,000–10,000/mm³ Pregnancy/labor/postpartum: may be elevated Labor/postpartum: around 15,000 can be expected	Do not diagnose infection from WBC alone postpartum. Look for fever, foul-smelling lochia, uterine tenderness, chills, tachycardia, or wound drainage.
Platelets	150,000–400,000/mm³	Low platelets increase bleeding risk and may indicate preeclampsia/HELLP. May affect epidural eligibility.
Blood Type / ABO	A, B, AB, or O	Important for transfusion needs, especially with postpartum hemorrhage.
Rh Factor	Rh-positive or Rh-negative	Rh-negative mothers may need RhoGAM at 28 weeks and within 72 hr postpartum if baby is Rh-positive.
Antibody Screen / Indirect Coombs	Negative	Positive means possible sensitization. RhoGAM only helps if mother is not already sensitized.

Infection / Immunity Labs

Lab	Basic Value	Nursing Cue
Rubella Titer	Immune/positive	If non-immune, give vaccine postpartum. Do not give during pregnancy because it is live.
Varicella Titer	Immune/positive	If non-immune, give vaccine postpartum. Do not give during pregnancy because it is live.
Hepatitis B / HBsAg	Negative	If mother is positive, newborn needs Hep B vaccine and HBIG after birth.
HIV	Negative	Antiretrovirals reduce transmission risk. Avoid breastfeeding in the U.S. if HIV-positive.
Syphilis / RPR or VDRL	Negative	Untreated syphilis can cause fetal complications. Treatment is usually penicillin.
Group B Strep	Negative	Positive GBS means IV antibiotics during labor to prevent newborn infection. Usually penicillin G or ampicillin.

Urine Labs

Lab	Basic Value	Nursing Cue
Urine Protein	Negative/trace	Proteinuria with hypertension suggests preeclampsia. Watch headache, vision changes, RUQ pain, and hyperreflexia.
Urine Glucose	Negative/trace	May suggest hyperglycemia or gestational diabetes risk. Follow up with glucose screening.
Urine Ketones	Negative	Can indicate dehydration, starvation, hyperemesis, or poor diabetes control.
Nitrites / Leukocytes	Negative	Positive findings suggest possible UTI. UTIs in pregnancy can trigger preterm labor.

Gestational Diabetes Labs

Lab	Basic Value	Nursing Cue
1-Hour Glucose Challenge	Usually less than 130–140 mg/dL	Screening test for gestational diabetes. Usually done at 24–28 weeks. Elevated result means 3-hour test is needed.
3-Hour Glucose Tolerance Test	Should stay below pregnancy glucose cutoffs during fasting, 1-hr, 2-hr, and 3-hr checks	Diagnostic test for gestational diabetes. GDM increases risk for macrosomia, shoulder dystocia, neonatal hypoglycemia, C-section, and preeclampsia.

Preeclampsia / HELLP Labs

Lab	Basic Value	Nursing Cue
AST	About 10–40 units/L	Elevated AST can indicate liver involvement in preeclampsia/HELLP.
ALT	About 7–56 units/L	Elevated ALT can indicate liver involvement in preeclampsia/HELLP.
Creatinine	About 0.6–1.1 mg/dL	Elevated creatinine can indicate kidney involvement.
BUN	About 7–20 mg/dL	Elevated BUN can indicate kidney involvement or dehydration.
LDH	About 140–280 units/L	Elevated LDH can indicate hemolysis. Think HELLP.
Uric Acid	About 2.5–6 mg/dL	May be elevated in preeclampsia and can support concern for worsening disease.
Platelets	150,000–400,000/mm³	Low platelets are part of HELLP and increase bleeding risk.
Urine Output	At least 30 mL/hr	Less than 30 mL/hr is concerning, especially with preeclampsia or magnesium sulfate therapy.

Magnesium Sulfate Monitoring

Lab / Assessment	Basic Value	Nursing Cue
Magnesium Level	Therapeutic: 4–7 mEq/L	Used for seizure prevention in preeclampsia.
Toxicity Signs	Too high = toxicity	Watch for loss of DTRs, respiratory depression, decreased urine output, extreme weakness, or lethargy.
Antidote	Calcium gluconate	Keep available when patient is receiving magnesium sulfate.

ROM / Preterm Labor Tests

Test	Basic Result	Nursing Cue
Nitrazine Test	Amniotic fluid turns paper blue	Tests vaginal fluid pH. Positive may mean rupture of membranes. False positives include blood, semen, and infection.
Fern Test	Fern-like pattern if positive	Used to confirm rupture of membranes.
Fetal Fibronectin	Negative is reassuring	Negative means delivery is unlikely soon. Positive means possible increased risk for preterm birth but is less specific.

Trauma / Bleeding Test

Test	Basic Result	Nursing Cue
Kleihauer-Betke	Minimal/none fetal blood in maternal circulation	Used after trauma, bleeding, or suspected fetomaternal hemorrhage. Helps determine if extra RhoGAM is needed.

Newborn Labs

Lab	Basic Value	Nursing Cue
Newborn Blood Glucose	Usually should be above 40–45 mg/dL	Check if newborn is jittery, lethargic, cold, poor feeding, infant of diabetic mother, LGA, SGA, or preterm.
Newborn Bilirubin	Rises after birth and usually peaks around day 3–5 in term newborns	Bilirubin should rise, peak, then gradually decrease as baby feeds and stools. Jaundice in the first 24 hr is concerning/pathologic. Phototherapy may be ordered.
Direct Coombs	Negative	Positive means antibodies are attached to newborn RBCs, increasing risk for hemolysis and jaundice.
Newborn Blood Type / Rh	Depends on baby	Helps identify ABO/Rh incompatibility and determines if Rh-negative mother needs postpartum RhoGAM.